Reg. No. RMEE2552994
PURE PATH LAB
Add: Infront of Cantabil, Beside Distt. Hospital-Gonda
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PURE PATH LAB
Palce of Accuracy & Your Satisfaction
Dr. Abha Mishra
M.B.B.S (M.D. PATH)
Reg. No-MP-8779

Mr. Shiv Prasad
Age / Sex : 58 YRS / Male
Reg. no. : 7004

Registered on : -
Collected on : -
Received on : -
Reported on : -

HAEMATOLOGY

TEST		VALUE	UNIT	REFERENCE
HEMOGLOBIN		13.0	g/dl	13 - 17
TOTAL LEUKOCYTE COUNT		7200	cumm	4,800 - 10,800
NEUTROPHILS		65	%	40 - 80
LYMPHOCYTE		29	%	20 - 40
EOSINOPHILS		04	%	1-6
MONOCYTES		02	%	2 - 10
BASOPHILS		00	%	<2
PLATELET COUNT	**L**	**1.27**	**lakhs/cumm**	**1.5 - 4.1**
TOTAL RBC COUNT	**L**	**3.77**	**million/cumm**	**4.5 - 5.5**
HEMATOCRIT VALUE, HCT	**L**	**33.2**	**%**	**40 - 50**
MEAN CORPUSCULAR VOLUME, MCV		88.1	fL	83 - 101
MEAN CELL HAEMOGLOBIN, MCH	**H**	**34.5**	**Pg**	**27 - 32**
MEAN CELL HAEMOGLOBIN CON, MCHC	**H**	**39.2**	**g/dl**	**31.5 - 34.5**
MEAN PLATELET VOLUME, MPV		10.6	fL	6.5 - 12
R.D.W. - SD		43.2	fL	39 - 46
R.D.W. - CV	**H**	**14.2**	**%**	**11.6 - 14**
P-LCR		32.5	19.7	- 42.4
P.D.W.	**H**	**16.3**	**fL**	**9.6 - 15.2**

Possible causes of abnormal parameters

	High	Low
RBC, Hb, or HCT	Dehydration, polycythemia, shock, chronic hypoxia	Anemia, thalassemia, and other hemoglobinopathies
MCV	Macrocytic anemia, liver disease	Microcytic anemia
WBC	Acute stress, infection, malignancies	Sepsis, marrow hypoplasia
Platelets	Risk of thrombosis	Risk of bleeding

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BIOCHEMISTRY

TEST	VALUE	UNIT	REFERENCE
RH (ANTI -D)	POSITIVE		
SERUM BILIRUBIN (TOTAL)	0.98	mg/dl	0.2 - 1.2
SERUM BILIRUBIN (DIRECT)	0.18	mg/dl	0 - 0.3
SERUM BILIRUBIN (INDIRECT)	0.80	mg/dl	0.2 - 1
SERUM CREATININE	1.41	mg/dl	0.72 - 1.43
SERUM UREA	38.54	mg/dl	18 - 55
SGPT (ALT)	26.87	U/l	13 - 40
SGOT (AST)	32.1	U/l	0 - 37
RANDOM BLOOD SUGAR	100.5	mg/dl	70 - 140

Serum Bilirubin (Indirect) - Description

Bilirubin is the orange-yellow pigment derived from the breakdown of hemoglobin (heme). The majority of bilirubin comes from senescent red cells. It is biotransformed in the liver and excreted in bile and urine. Some conjugated bilirubin is bound to serum albumin, so-called D (delta) bilirubin.

Serum Creatinine - Interpretation

Creatinine is a waste product produced by muscles from the breakdown of a compound called creatine. It is removed from the body by the kidneys, which filter almost all of it from the blood and release it into the urine. Creatine is part of the cycle that produces energy needed to contract muscles. Both creatine and creatinine are produced by the body at a relatively constant rate. Since almost all creatinine is filtered from the blood by the kidneys and released into the urine, blood levels are usually a good indicator of how well the kidneys are working. Higher than normal level may be due to: - Blockage in the urinary tract - Kidney problems, such as kidney damage or failure, infection, or reduced blood flow - Loss of body fluid (dehydration) - Muscle problems, such as breakdown of muscle fibers - Problems during pregnancy, such as seizures (eclampsia)), or high blood pressure caused by pregnancy (preeclampsia) Lower than normal level may be due to:- Myasthenia Gravis - Muscular dystrophy.Low serum creatinine values are rare. They almost always reflect low muscle mass.

SGPT (ALT) - Interpretation

Physiological basisIntracellular enzyme involved in amino acid metabolism. Present in large concentrations in liver, kidney; in smaller amounts, in skeletal muscle and heart. Released with tissue damage, particularly liver injury.Interpretation Increased in: Acute viral hepatitis (ALT > AST), biliary tract obstruction (cholangitis, choledocholithiasis), alcoholic hepatitis and cirrhosis (AST > ALT), liver abscess, metastatic or primary liver cancer; nonalcoholic steatohepatitis; right heart failure, ischemia or hypoxia, injury to liver (“shock liver”), extensive trauma; drugs that cause cholestasis or hepatotoxicity.Decreased in: Pyridoxine (vitamin B6) deficiency.Comments ALT is the preferred enzyme for evaluation of liver injury.

SGOT (AST) - Interpretation

Physiological basis:Intracellular enzyme involved in amino acid metabolism. Present in large concentrations in liver, skeletal muscle, brain, red cells, and heart. Released into the bloodstream when tissue is damaged, especially in liver injury.Interpretation Increased in: Acute viral hepatitis (ALT > AST), biliary tract obstruction (cholangitis, choledocholithiasis), alcoholic hepatitis and cirrhosis (AST > ALT), liver abscess, metastatic or primary liver cancer; right heart failure, ischemic or hypoxic injury to liver (“shock liver”), extensive

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trauma. Drugs that cause cholestasis or hepatotoxicity.Decreased in: Pyridoxine (vitamin B6) deficiencyComments Test is not indicated for diagnosis of myocardial infarction.AST/ALT ratio >1 suggests cirrhosis in patients with hepatitis C

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SEROLOGY & IMMUNOLOGY

TEST	VALUE	UNIT	REFERENCE
HIV - 1	NEGATIVE		
HIV - 2	NEGATIVE		
HEPATITIS C VIRUS, HCV	NEGATIVE		
HBSAG	NEGATIVE		

HbsAg

HbsAg is the first serologic marker appearing in the serum 6-16 weeks following hepatitis B viral infection. In typical HBV infection, HBsAg will be detected 2-4 weeks before the liver enzyme levels (ALT) become abnormal and 3-5 weeks before the patient develops jaundice. In acute cases HbsAg usually disappears 1-2 months after the onset of symptoms. Persistence of HbsAg for more than 6 months indicates development of either a chronic carrier state or chronic liver disease. The presence of HbsAg is frequently associated with infectivity. HbsAg when accompanied by Hepatitis Be antigen and/or hepatitis B viral DNA almost always indicates infectivity.

Anti HCV card test interpretation

Reactive test result indicates presence of Hepatitis C virus infection. Active infection to be confirmed by HCV RNA PCR test. It cannot differentiate between the stages of Hepatitis C viral infection nor used to monitor the efficacy of treatment. Non-Reactive test result indicates Hepatitis C virus infection is unlikely.

HIV Interpretation

A negative result implies that no Anti HIV-1 & HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the “WINDOW PHASE” (before the development of detectable levels of antibodies). A positive result suggests the possibilities of HIV-I and / or HIV-II infection. However these results must be verified by a confirmatory test (IFA / WESTERN BLOT I-II) before pronouncing the patient positive for HIV-1 and / or HIV-2 infection. All reactive samples should be confirmed by using HIV Western Blot/PCR.

~~~ End of report ~~~
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